PHARMACOVIGILANCE INTERNSHIP

TASK 2

ADR REPORTING & DOCUMENTATION

Structured Case Report and Reporting-Form Simulation

Prepared by	Shivya Singh
Role	Pharmacovigilance Intern
Task type	Fictional educational training case
Case focus	Suspected ibuprofen-associated hypersensitivity-type reaction
Reporting format	Structured ADR documentation + assessment narrative

Educational scope
This document converts the Task 1 fictional case into a more structured Task 2 ADR-reporting format. It separates observed facts from preliminary interpretation and explicitly marks information that was not provided, rather than inventing it.

CONFIDENTIALITY & TRAINING DECLARATION

The patient scenario, clinical details and reporter presentation are fictional and intended only for internship portfolio practice. This is not a real patient record, medical advice, or a substitute for an approved pharmacovigilance reporting process.

1. CASE PROFILE

The Task 1 scenario describes a 35-year-old woman with intermittent migraine and seasonal allergic symptoms. She was already taking cetirizine 10 mg once daily. Ibuprofen 400 mg three times daily after food was newly started for bacterial pharyngitis. On the second day, an itchy, raised rash appeared over both forearms and upper chest, with mild swelling around the lips. No new food, cosmetic product, insect exposure or other newly started medicine was reported. Ibuprofen was discontinued, symptoms began improving within approximately 24 hours and later settled. Rechallenge was not performed.

Section	Field	Recorded case information
Patient identifier	**Initials / identifier**	Not provided in the source case; fictional case only.
Demographics	**Age / sex**	35 years / female
Relevant history	**Background condition(s)**	Intermittent migraine; seasonal allergic symptoms
Concomitant medicine	**Current medicine**	Cetirizine 10 mg once daily
Suspected medicine	**Name / strength**	Ibuprofen 400 mg
Suspected medicine	**Dose / frequency / route**	400 mg three times daily after food; route not explicitly stated in source
Indication	**Reason for treatment**	Bacterial pharyngitis
Therapy timeline	**Exposure anchor**	Day 0: ibuprofen initiated; Day 2: discontinued
Reaction	**Description**	Pruritic raised rash over both forearms and upper chest, with mild lip swelling
Outcome	**Course**	Improvement began within ~24 hours after withdrawal; reaction subsequently settled
Rechallenge	**Re-exposure**	Not performed

Reporting-quality note

The source case provides relative timing rather than calendar dates. Therefore this report keeps the Day 0 / Day 2 timeline rather than creating dates that were not supplied. Similarly, no patient initials, laboratory results, exact recovery date, or reporter contact information are added.

2. STRUCTURED ADR REPORTING FORM - SIMULATION

The following form is a portfolio-style transcription of the available case facts into common ADR-reporting fields. Entries marked "not provided" should be completed from the original clinical record in a real-world report.

Section	Field	Recorded case information
Patient information	**Patient identifier**	Not provided - fictional educational case
Patient information	**Age / sex**	35 years / female
Patient information	**Weight**	Not provided
Patient information	**Relevant history**	Intermittent migraine; seasonal allergic symptoms
Suspected medicine	**Medicine**	Ibuprofen 400 mg
Suspected medicine	**Indication**	Bacterial pharyngitis
Suspected medicine	**Regimen**	400 mg three times daily after food
Suspected medicine	**Start / stop**	Day 0 start; Day 2 stopped
Reaction	**Event term**	Pruritic raised rash with mild lip swelling
Reaction	**Anatomical sites**	Both forearms and upper chest; lips
Reaction	**Onset**	Day 2 after ibuprofen initiation
Reaction	**Seriousness**	Not stated in the source case
Reaction	**Outcome**	Improved after withdrawal; subsequently settled
Management	**Action on suspected medicine**	Ibuprofen discontinued
Management	**Treatment for reaction**	No additional treatment stated in the source case
Dechallenge	**Response after withdrawal**	Improvement began within approximately 24 hours
Rechallenge	**Re-exposure**	Not performed
Alternative causes	**Exposure review**	No new food, cosmetic, insect exposure or other newly started medicine reported
Concomitant medicine	**Medicine**	Cetirizine 10 mg once daily; already ongoing
Investigations	**Relevant tests**	Not provided
Reporter	**Prepared / reported by**	Shivya Singh - Pharmacovigilance Intern (educational project preparer)
Reporter	**Contact / signature**	Not provided

Field-level interpretation
The structured record identifies ibuprofen as the suspected medicine because it was the new exposure immediately preceding the reaction. The report records the positive dechallenge signal but does not claim that this alone proves causality.

3. CASE NARRATIVE & TIMELINE

Narrative

A 35-year-old woman with intermittent migraine and seasonal allergic symptoms was receiving cetirizine 10 mg once daily. Ibuprofen 400 mg three times daily after food was initiated for bacterial pharyngitis. On Day 2, she developed an itchy, raised rash involving both forearms and the upper chest, accompanied by mild swelling around the lips. The case report states that no new food, cosmetic product, insect exposure or other newly started medicine was identified. Ibuprofen was discontinued on Day 2. Symptoms began improving within approximately 24 hours and subsequently settled. No rechallenge was performed.

Event timeline

Time	Exposure / event	Reporting relevance
Day 0	Ibuprofen 400 mg TID after food initiated for bacterial pharyngitis.	New exposure establishes the principal temporal anchor.
Day 2	Pruritic raised rash on both forearms and upper chest; mild lip swelling.	Reaction follows recent medicine initiation.
Day 2	Ibuprofen discontinued.	Dechallenge observation begins.
~24 h later	Itching and rash begin improving.	Positive dechallenge signal supports a suspected association.
Later	Reaction settles; no rechallenge.	Outcome documented; no re-exposure evidence available.

Event characterization

Observed event	Pruritic, raised rash with mild lip swelling.
Suspected ADR pattern	Hypersensitivity-type presentation; Type B in the Task 1 educational classification.
Temporal relationship	Compatible: reaction occurred about two days after new ibuprofen exposure.
Dechallenge	Positive: symptoms started improving after ibuprofen was stopped.
Rechallenge	Not performed; therefore no re-exposure evidence is available.
Alternative explanations	No competing new exposure was reported in the fictional scenario.

4. CAUSALITY & REPORTING ASSESSMENT

Preliminary causality assessment

Assessment factor	Case evidence	Interpretation
Temporal relationship	Reaction appeared approximately two days after ibuprofen initiation.	Supports a temporal association.
Dechallenge	Symptoms began improving after ibuprofen was discontinued.	Supports the suspected relationship.
Rechallenge	Not performed.	No additional confirmation from re-exposure.
Alternative causes	No new food, cosmetic, insect exposure or other newly started medicine reported.	No clear competing new trigger documented in the scenario.
Concomitant medicine	Cetirizine was already being taken.	Relevant background medicine; real-case review would still require full medication history.

Preliminary conclusion: The event is consistent with a suspected ibuprofen-associated hypersensitivity-type ADR. The timing and positive dechallenge support the assessment, while the absence of rechallenge and the limited fictional case data prevent a definitive causality conclusion.

Report completeness / missing-data check

Item	Status in source case	Action for a real report
Patient identifier / initials	Not provided	Use approved non-identifying patient information.
Calendar dates	Not provided; only Day 0 / Day 2 timing	Enter exact treatment and onset dates from the record.
Investigations	Not provided	Add relevant examination or test results if available.
Seriousness criteria	Not stated	Record applicable seriousness criteria in the real form.
Reporter contact / signature	Not provided	Complete the authorized reporter fields.

5. REPORTING WORKFLOW & QUALITY CONTROL

Suggested reporting workflow for this educational case

Step	Action	Application to this case
1	**Identify**	Recognize the suspected reaction and define the event in clinical terms.
2	**Document**	Capture patient, medicine, reaction, timeline, management and outcome information.
3	**Assess**	Review timing, dechallenge, rechallenge, concomitant medicines and alternative causes.
4	**Submit**	Use the appropriate ADR reporting channel / monitoring-centre process in the relevant setting.
5	**Evaluate**	Support ongoing safety assessment, data analysis and signal evaluation where applicable.

Quality-control checklist

[OK] Observed reaction described separately from causality interpretation.

[OK] Suspected medicine, dose, frequency and treatment timing documented from the source case.

[OK] Dechallenge and rechallenge are recorded as separate fields.

[OK] Alternative new exposures were explicitly reviewed as stated in the scenario.

[OK] Information not supplied by the case is marked "not provided" rather than fabricated.

[OK] Fictional educational status is visible throughout the report.

[OK] No YouTube or external video link is included in the submission document.

Key learning points

- A suspected ADR can be documented before causality is established with certainty.

- A clear exposure-to-onset timeline is central to patient-level assessment.

- Improvement after withdrawal can support the suspected relationship but is not definitive proof.

- Type B reactions in the Task 1 framework are generally unpredictable and may include hypersensitivity patterns.

- Good pharmacovigilance documentation distinguishes known facts, missing data and preliminary interpretation.

FINAL EDUCATIONAL DECLARATION

This Task 2 document is an original, portfolio-oriented transformation of the fictional Task 1 scenario supplied for the internship. It is not a real ADR report, contains no real patient identifiers, and should not be used for clinical decision-making. Any real-world pharmacovigilance submission must be completed using verified patient and reporter information and the applicable reporting procedure.

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